



FREE GUIDE · FOR HOSPITALS & HEALTH SYSTEMS

The Hospital's Guide to Compliant Waste Disposal.

Every regulated stream, every department —
centralized, documented, defensible.

DEA-REGISTERED

EPA-COMPLIANT

NO CONTRACT

CERTIFICATE OF DESTRUCTION

Compliance, made simple.

Every stream, at scale.

Hospitals and health systems generate every regulated waste stream at scale, across dozens of departments — controlled substances, sharps, red-bag biohazard, pharmaceutical, RCRA-hazardous, and chemotherapy. The challenge isn't any one stream; it's centralized, documented, defensible management across the whole system. This guide covers each stream.

The good news

One DEA-registered partner can centralize every stream — with non-retrievable controlled destruction, Form 222/41, USP 800 and RCRA handling, and audit-ready Certificates of Destruction. Scheduled pickup and multi-site programs available.

AT A GLANCE

Who regulates what.

Agency	What it covers in your practice
OSHA	Bloodborne Pathogens Standard — sharps handling, containers, and annual staff training.
EPA	RCRA — hazardous pharmaceutical waste identification, segregation, and destruction.
DEA	Controlled substances — non-retrievable destruction and DEA Form 41 documentation.
DOT	Safe shipping of regulated medical and pharmaceutical waste in transit.
State	Health-department and environmental rules that vary by state.

THE STREAMS

Every stream, in depth.

Below is each regulated stream your practice produces — what belongs in it, what does not, the rule behind it, how to dispose of it, and a tip to stay clean.

WASTE STREAM 01

Controlled substances

Schedule II–V across pharmacy, OR, ED, and floors — the highest diversion-risk stream.

GOES IN THIS STREAM

- Expired / unused Schedule II–V stock
- Wasted & partial doses
- Surrendered controlled stock

DOES NOT GO HERE

- Non-controlled meds (pharmaceutical kit)
- Sharps

THE RULE · DEA — render non-retrievable, Form 222 for Schedule II transfers, Form 41 for surrendered controls, witnessed logs.

DISPOSE WITH · Controlled-substance destruction, to DEA standards.

Tip

Diversion risk makes witnessed wasting logs and a clean paper trail non-negotiable.

WASTE STREAM 02

Sharps

Enormous volume across every department.

GOES IN THIS STREAM

- Needles, syringes & lancets
- Surgical & procedure sharps
- IV & blood-draw sharps

DOES NOT GO HERE

- Bloody gauze (biohazard)
- Expired meds (pharmaceutical)
- Non-sharp packaging

THE RULE · OSHA Bloodborne Pathogens Standard — FDA-cleared, puncture-resistant containers, no overfilling, annual training.

DISPOSE WITH · Sharps mail-back kit or scheduled pickup.

Tip

Replace containers at the fill line; high-volume sites should check daily.

WASTE STREAM 03

Biohazard / regulated medical waste

Red-bag regulated medical waste, system-wide.

GOES IN THIS STREAM

- Blood-soaked materials & drapes
- Contaminated PPE
- Specimen & culture waste

DOES NOT GO HERE

- Sharps (sharps kit)
- Lightly soiled everyday items
- Expired meds (pharmaceutical)

THE RULE · OSHA plus your state's medical-waste rules — red bags, labeling, treatment/destruction, retained documentation.

DISPOSE WITH · Biohazard mail-back kit or scheduled pickup.

Tip

Only visibly blood/OPIM-contaminated items are red-bag waste — over-classifying at volume is costly.

WASTE STREAM 04

RCRA-hazardous waste

P/U-listed and characteristic hazardous pharmaceutical waste.

GOES IN THIS STREAM

- P-listed drugs (warfarin, epinephrine)
- U-listed & characteristic waste
- Hazardous chemicals

DOES NOT GO HERE

- Non-hazardous pharmaceuticals
- Controlled substances (own path)

THE RULE · EPA RCRA — identify P/U-listed and characteristic waste, segregate, and destroy to EPA standards; know your generator status.

DISPOSE WITH · RCRA-hazardous mail-back kit or pickup.

Tip

At a facility scale, generator status and segregation drive both compliance and cost.

WASTE STREAM 05

Trace & bulk chemotherapy (USP 800)

Trace and bulk chemotherapy from oncology and infusion.

GOES IN THIS STREAM

- Trace (RCRA-empty) chemo waste
- Bulk / unused hazardous drug
- Contaminated USP 800 PPE

DOES NOT GO HERE

- Non-hazardous pharmaceuticals
- Sharps (sharps kit)

THE RULE · USP 800 and EPA RCRA — segregate trace (RCRA-empty) from bulk hazardous drug; incinerate/destroy to the required standard.

DISPOSE WITH · Trace chemo mail-back kit; bulk chemo scoped as RCRA-hazardous.

Tip

Trace (RCRA-empty) and bulk chemo follow different paths — segregate them.

WASTE STREAM 06

Pharmaceutical waste

Expired non-controlled, non-hazardous medications.

GOES IN THIS STREAM

- Expired non-controlled Rx
- Non-controlled injectables
- Samples

DOES NOT GO HERE

- Controlled substances (own kit)
- RCRA-hazardous drugs (own kit)
- Sharps

THE RULE · Do not drain-dispose; segregate hazardous and controlled drugs; destroy with documentation.

DISPOSE WITH · Pharmaceutical waste mail-back kit or pickup.

Tip

Segregate hazardous and controlled drugs first — everything else routes here.

YOUR CHECKLIST

The hospital compliance checklist.

☐ Controlled substances centrally logged, wasted-dose witnessed, non-retrievable with Form 222/41.

☐ Sharps program standardized across departments; containers never overfilled.

- ☐ Red-bag biohazard segregated correctly (over-bagging at scale is costly).
- ☐ RCRA-hazardous drugs (P/U-listed) identified, segregated, generator status managed.
- ☐ Trace vs bulk chemo segregated per USP 800.
- ☐ Audit-ready Certificates of Destruction archived system-wide.

QUESTIONS HOSPITALS ASK

Your hospital questions, answered.

Can you centralize disposal across our whole system?

Yes — a single program across departments and sites, with centralized documentation and Certificates of Destruction, is exactly what we build for health systems.

How do you support controlled-substance diversion controls?

Non-retrievable destruction with witnessed wasting logs and Form 222/41, plus chain-of-custody — auditable across the system.

Do you handle USP 800 chemo and RCRA-hazardous waste?

Yes — trace and bulk chemo per USP 800, and P/U-listed and characteristic hazardous drugs to EPA standards.

Mail-back or pickup?

Scheduled pickup for a hospital's volume, with mail-back for outlying clinics where it fits.

THE EASY WAY

Handle it with mail-back.

Easy Rx Cycle builds a centralized program for hospitals and health systems — scheduled pickup for volume, mail-back for outlying sites — covering every stream with non-retrievable controlled destruction, USP 800 and RCRA handling, and audit-ready Certificates of Destruction.

QUICK REFERENCE · WHICH KIT FOR WHICH STREAM

Waste stream	The right path
Controlled substances	Controlled destruction (Form 222/41, chain-of-custody)
Sharps	Pickup or kits, standardized
Biohazard / RMW	Pickup or kits

Waste stream	The right path
RCRA-hazardous drugs	RCRA-hazardous destruction
Chemo (trace/bulk)	Trace kit / bulk RCRA (USP 800)
Non-hazardous Rx	Pharmaceutical waste

Prefer pickup, or higher volume?

Hospital volume calls for a centralized, scheduled program with system-wide records. Let's scope it — get a quote at easyrxcycle.com/get-a-quote or call 501-904-2929.

WHY HOSPITALS CHOOSE EASY RX CYCLE

- No contract, no minimums — buy as you need
- Flat-rate pricing — no fuel or hidden surcharges
- One vendor for every stream you generate
- DEA-registered, EPA- & OSHA-aligned destruction
- A Certificate of Destruction on every single order

Ready to simplify your hospital waste?

Shop mail-back kits with prices, or get a custom quote for pickup and higher volume. No contract, cancel anytime.

501-904-2929 · sales@easyrxcycle.com · easyrxcycle.com/shop